INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Gemcitabine + carboplatin AUC2

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00430
Version 6

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

Treatment stages

01 **Gemcitabine + carboplatin AUC2**
21-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	11
04	Sources and further information	17

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Gemcitabine + carboplatin AUC2
National Cancer Control Programme (NCCP) 00430

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for fever, shivering, unusual bleeding, severe mouth or bowel symptoms, inability to drink, reduced urine, new breathlessness or a painful/leaking line. Severe chest pain, collapse or severe breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

Control advanced cancer and symptoms, with regular review of benefit and side effects.

PLATINUM CHEMOTHERAPY

Carboplatin



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) inside cells

Carboplatin creates links in DNA, making it harder for cells to copy and repair their genetic instructions. Its dose depends on kidney function as well as the intended regimen.

Platinum enters cell → DNA links form → Copying is disrupted

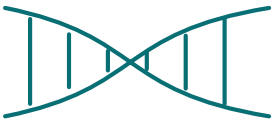
What to watch for

Keep blood-test appointments. Fever, unusual bleeding, repeated vomiting or infusion symptoms need prompt advice.

ANTIMETABOLITE CHEMOTHERAPY

Gemcitabine

intravenous (IV) infusion



Interferes with DNA synthesis

Gemcitabine is converted inside cells into forms that disrupt DNA building blocks and DNA copying. It can affect normal blood-cell production as well as cancer cells.

Activated inside cells → DNA production disrupted → Cell division slowed

What to watch for

Report fever, unusual bruising, breathlessness, swelling or reduced urine urgently. Flu-like symptoms cannot safely be assumed to be a harmless drug effect.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

Gemcitabine with carboplatin is an option for locally recurrent or metastatic triple-negative breast cancer. Previous medicines, cancer activity, symptoms, fitness, blood-count recovery, kidney function and available biomarker-directed options guide the choice.

Your exact schedule

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

Alternatives

Discuss suitable endocrine, targeted or chemotherapy options, expected benefit, practical demands and the option of supportive care according to your setting.

Why this treatment may be chosen

Show trial results and what the numbers mean

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit, alternatives and uncertainty for your own cancer.

CHAPTER 02

Your schedule, tests and appointments

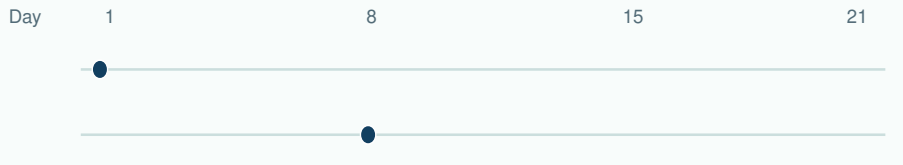
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Gemcitabine + carboplatin AUC2 · 21-day reference

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY REFERENCE

Gemcitabine + carboplatin AUC2

21-day interval

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

These are the medicines in this exact schedule. Dose holds, missed tablets and omitted visits need instructions from your treating team.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00430 v6 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Gemcitabine + carboplatin AUC2: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00430	Confirm exact regimen	Gemcitabine + carboplatin AUC2	Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00430	Locally recurrent or metastatic triple-negative breast cancer	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00430	full blood count (FBC), renal and liver profile	Day 1: FBC, renal and liver tests	Day 8: FBC; review both doses	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00430	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/ breathing symptoms	Clinical indication
Cancer review NCCP 00430 · METASTATIC · MARKERS	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks NCCP 00430 · BONE	Review previous treatment	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1

Baseline review

Before treatment

Locally recurrent or metastatic triple-negative breast cancer. There is no routine three-month echo schedule for CMF or gemcitabine alone. Review prior treatment and cardiac risk; new chest pain or breathing symptoms need urgent assessment.

Ask / act: Confirm source eligibility and results.

2 **Treatment-day check**

Before each planned dose

Baseline and Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Review kidney-function dosing and whether either medicine is held before each administration.

Ask / **act:** Confirm active medicines and whether a dose is held.

3 **Support plan**

At every phase or day change

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

Ask / **act:** Choose the matching support script; do not merge phases.

4 **Response and follow-up**

Agreed review points

For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Ask / **act:** Review symptoms, results and patient priorities together.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the first visit

Blood tests, medicines review and the consent discussion come first. Review previous treatment, organ function and the treatment-day blood-test plan.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

Allow time for blood review, preparation, treatment and observation. Your unit confirms the full visit duration. Arrange transport or an escort where advised.

Phase-specific visit pattern

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

Checks across treatment

Your clinical team books tests, reviews results and explains any change. Contact them if an expected test or instruction is missing.

Blood and kidney tests

Baseline and Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Review kidney-function dosing and whether either medicine is held before each administration.

Heart and breathing

There is no routine three-month echo schedule for CMF or gemcitabine alone. Review prior treatment and cardiac risk; new chest pain or breathing symptoms need urgent assessment.

Cancer imaging

For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Bone health

Bone-density testing follows endocrine treatment, ovarian suppression, menopause and other bone-risk factors. dual-energy X-ray absorptiometry (bone density) (DEXA) is not automatically required before every chemotherapy course.

The wider treatment plan

For advanced cancer, review response and toxicity to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

When treatment changes

Reconcile active cancer medicines, supportive medicines and remaining symptoms before every transition.

Follow-up

Keep the agreed review appointments and report new symptoms between visits; this map does not detect missed tests.

Hormone treatment and bone care

Hormone-receptor results and your treatment setting determine whether endocrine therapy is appropriate. It has its own monitoring plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Gemcitabine + carboplatin AUC2 · 21-day reference

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment · Day 1	 **Day 1** Treatment in the unit
IV treatment · Day 8	 **Day 8** Treatment in the unit
Blood counts / infection	 **Days 1–21** Fever needs urgent advice on any day
Fatigue	 **Days 1–21** May build across cycles
Nausea / appetite	 **Days 1–21** Follow the written prevention plan
Breathing / swelling	 **Days 1–21** New breathlessness or reduced urine: seek advice

General orientation: Symptoms can occur at any time, including days without a treatment visit. The graph does not predict symptom-free days.

Treatment days: A delayed or omitted dose needs an updated plan from the unit. Do not move doses yourself.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00430 v6 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–21: One reference cycle

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity. Use your written appointment and medicine plan; no dates are entered here.

Support around the medicines

1 Anti-sickness plan

Follow the prevention and rescue instructions prescribed for this schedule. Call if vomiting prevents drinking.

2 Infection and bleeding

Know the local urgent-contact number. Fever, shivering, sudden illness or unusual bleeding needs prompt advice even between visits.

3 Medicine review

Show the oncology pharmacist all prescribed, over-the-counter and herbal medicines. Ask before starting anything new.

4 Hydration and symptoms

Agree fluid intake with the team, especially with heart or kidney fluid restrictions. Report reduced urine, swelling, mouth ulcers or diarrhoea.

Questions worth asking

Who arranges the next tests?

Your treating service arranges and reviews tests. If instructions or appointments are missing, contact the team; the map does not book tests.

What if a dose is missed or I feel unwell?

Contact the unit for a revised plan. Do not double tablets, shift treatment dates or assume that an omitted dose should be made up.

Try explaining the plan back: Which medicines are due on which days? What symptoms mean you should call today? Which tests come next and who arranges them?

Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

Gemcitabine

- Flu-like symptoms, fatigue, rash and swelling
- Uncommon serious lung or liver injury
- Rare haemolytic uraemic syndrome and kidney injury

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Gemcitabine + carboplatin AUC2

Active medicines: Carboplatin + Gemcitabine

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before chemotherapy on Day 1 Review QT risk, constipation, hepatic function and any alternative local regimen.
Dexamethasone	8 mg orally once daily	Days 1, 2 and 3 Reconcile total steroid exposure; individual adjustments require prescribing review.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Carboplatin is AUC2 on BOTH Days 1 and 8; do not use an AUC4/5 Day 1-only template. The AUC≥4 antiemetic category does not apply to this AUC2 schedule.
- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · Gemcitabine + carboplatin AUC2

Active medicines: Carboplatin + Gemcitabine

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before chemotherapy on Day 8 Review QT risk, constipation, hepatic function and any alternative local regimen.
Dexamethasone	8 mg orally once daily	Days 8, 9 and 10 Reconcile total steroid exposure; individual adjustments require prescribing review.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be specified Review renal function, contraindications and interactions before prescribing.

Additional phase-specific support

- Confirm allergy/interaction review, blood tests and the source-specific renal/hepatic assessment before prescribing.
- Carboplatin is AUC2 on BOTH Days 1 and 8; do not use an AUC4/5 Day 1-only template. The AUC≥4 antiemetic category does not apply to this AUC2 schedule.
- Confirm the dose is going ahead before starting the linked support course. Reconcile rescue medicine duration rather than copying another treatment day.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Gemcitabine + carboplatin AUC2

Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood / infection

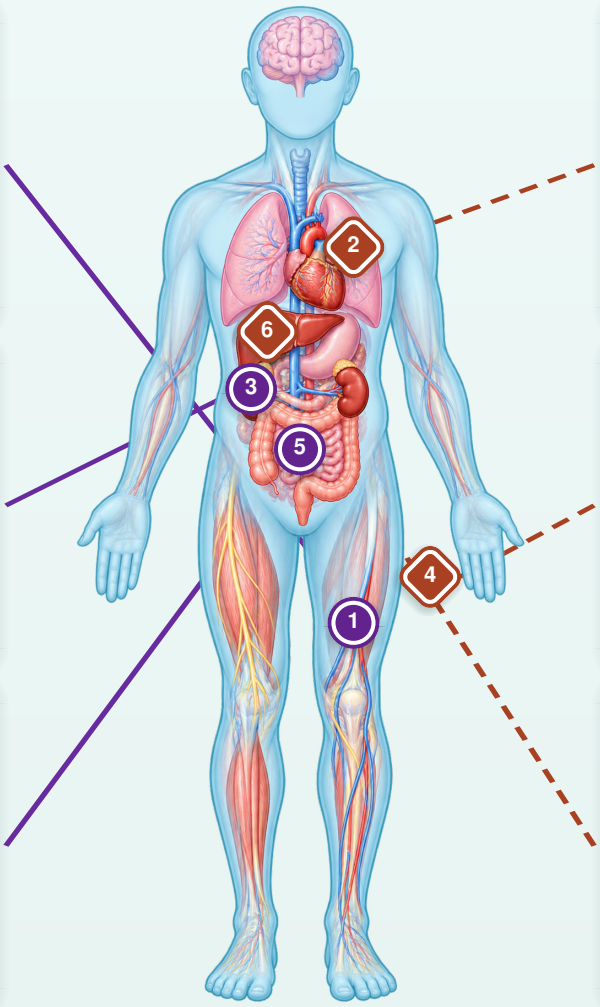
Gemcitabine / carboplatin

Report fever, unusual bruising or bleeding immediately.

2 Breathing

Gemcitabine

New cough or breathlessness needs prompt assessment.



3 Kidneys / urine

Gemcitabine / carboplatin

Report reduced urine, new swelling or sudden weakness.

4 Skin / swelling

Gemcitabine

Report a spreading rash, blistering or rapid swelling.

5 Nausea / appetite

Chemotherapy

Call if vomiting prevents drinking.

6 Liver

Gemcitabine

Report yellow skin or eyes, dark urine or severe abdominal pain.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Medicine-specific effects are highlighted; this is not a complete list. New or persistent symptoms still need review after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00430 v6 · official regimen**
   https://healthservice.hse.ie/documents/6832/430_GemcarboAUC2.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf